DEHU
PATHOLOGY LABORATORY

Your Health, Our Mission

At. Mahadwar Chowk,Near Primary Health Center, Dehu,Dist.Pune - 412109.

dehupathology897@gmail.com

9325251932

Patient Name : MR. ATTAR KASAM GULAB

Age / Gender : 76 Years / Male

Referral Doctor: DR.GOWANDE HOSPITAL

Collected At : DEHU PATHOLOGY LABORATORY

Collection Date : 10/11/2023 12:37 PM

Accession On : 10/11/2023 12:37 PM

Reporting Date : 10/11/2023 12:39 PM

Pt.Type / ID : Direct/ 4719

Complete Blood Count

Test Description	Value(s)	Unit	Reference Range
Hemoglobin	**11.7**	gms/dl	13 - 16
Erythrocyte(RBC) Count	4.52	mil./cmm	4.5 - 6.5
Haematocrit (HCT)	**32.6**	%	40 - 54
MCV (Mean Corpuscular Volume)	**72.1**	fL	80 - 100
MCH (Mean Corpuscular Hb)	**25.8**	pg	27 - 34
MCHC (Mean Corpuscular Hb Concn.)	35.8	gm/dl	32 - 36
RDW-CV (Red Cell Distribution Width)	**16.2**	%	11 - 16
Total Leucocytes (WBC) Count	**15600**	/uL	4000 - 10000
DIFFERENTIAL COUNT			
Absolute Neutrophils Count	**14040**	/c.mm	2000 - 7000
Absolute Lymphocyte Count	**624**	/c.mm	1000 - 3000
Absolute Monocyte Count	624	/c.mm	200 - 1000
Absolute Eosinophil Count	312	/c.mm	20 - 500
Neutrophils	**90**	%	40 - 70
Lymphocytes	**04**	%	20 - 40
Monocytes	04	%	2 - 8
Eosinophils	02	%	1 - 6
Basophils	00	%	0 - 1
Platelet Count	263000	/cmm.	150000 - 450000
MPV (Mean Platelet Volume)	9.2	fl	6 - 9.5

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WIDAL TEST AGGLUTINATION

Test Description	Value(s)	Unit	Reference Range
S. Typhi 'O'	**Agglutination 1:160**		No Agglutination
S. Typhi 'H'	**Agglutination 1:80**		No Agglutination
S.Paratyphi A(H)	No Agglutination		No Agglutination
S.Paratyphi B(H)	No Agglutination		No Agglutination
Result :	**Test is positive**		

Clinical significance:

- The organism Salmonella typhi responsible for causing enteric fever/typhoid fever, which is characterised by high consistent fever, loss of apetite, transitory bacteraemia, round or oval shaped ulcers on smooth peritoneal surface of peyer's patches etc.
- The organism possess 'O' antigen on cell wall and 'H' antigen on its flagella, against which the host body produces immunospecific antibodies.
- Paratyphoid fever caused by Salmonella paratyphi A or B is characterized by milder course of disease.
- These organisms also possess somatic 'O' and flagellar antigen termed as A(H) and B (H) respectively

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BLOOD GLUCOSE LEVEL (RANDOM)

Test Description	Value(s)	Unit	Reference Range
BLOOD SUGAR LEVEL(RANDOM) Plasma, Hexokinase	**65.10**	mg/dl	70 - 160

Note
• Whole blood glucose levels (capillary blood/ glucometer samples) are 12 - 15% lower than plasma concentrations.
• Exercise immediately before sample collection can lower random glucose test results.

Interpretation: • Conditions that can result in an elevated blood glucose level include: Acromegaly, Acute stress (response to trauma, heart attack, and stroke for instance), Chronic kidney disease, Cushing syndrome, Excessive consumption of food, Hyperthyroidism, Pancreatitis A low level of glucose may indicate hypoglycemia, a condition characterized by a drop in blood glucose to a level where first it causes nervous system symptoms (sweating, palpitations, hunger, trembling, and anxiety), then begins to affect the brain (causing confusion, hallucinations, blurred vision, and sometimes even coma and death). A low blood glucose level (hypoglycemia) may be seen with:Adrenal insufficiency, Drinking excessive alcohol, Severe liver disease, Hypopituitarism, Hypothyroidism, Severe infections, Severe heart failure, Chronic kidney (renal) failure, Insulin overdose, Tumors that produce insulin (insulinomas), Starvation

Clinical Utility: • Helpful in evaluation of diabetes and other carbohydrate metabolism disorders including gestational diabetes, neonatal hypoglycemia, idiopathic hypoglycemia and pancreatic islet cell carcinoma.

Associated Tests: • HbA1c , Diabetes Profile

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LIVER FUNCTION TEST (LFT)

Test Description	Value(s)	Unit	Reference Range
Serum Bilirubin, (Total) Diazo	**1.46**	mg/dL	0.2 - 1.2
Serum Bilirubin, (Direct) Diazo	**0.45**	mg/dL	0.0 - 0.3
Serum Bilirubin, (Indirect) Diazo	**1.01**	mg/dL	0.2 - 0.9
Aspartate Aminotransferase (AST/SGOT) IFCC	19.30	U/L	0 - 40
Alanine Aminotransferase (ALT/SGPT) IFCC	25.10	U/L	M 0 - 40
Alkaline Phosphatase (ALP)	201	U/L	80 - 360
Serum Total Protein (Biuret)	6.10	g/dL	6.0 - 8.3
Serum Albumin (Bromocresol green	4.30	g/dL	3.2 - 5.0
Serum Globulin	**1.80**	g/dL	2.5 - 3.3
Albumin/Globulin Ratio	**2.39**	-	1.0 - 2.1

Clinical Significance :
More common causes of elevated liver enzymes include: Medication, Alcohol abuse, Hepatitis A, B, C and E, Non Alcoholic fatty liver disease, Obesity,

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Dengue IgG & IgM Antibody (card Test)

Test Description	Value(s)	Unit	Reference Range
IgG Immunochromatography	Serum is Non Reactive		Serum is Non Reactive
IgM Immunochromatography	Serum is Non Reactive		Serum is Non Reactive

LIMITATIONS
1.The analysis of single serum sample should not be used as the sole criterion for diagnosis.
2. In early infection and some secondary infections, detectable levels of IgM antibodies may be low. Some patients may not produce detectable levels of antibody within the first seven to ten day after infection. Where symptoms persist, patients should be re-tested 3 - 4 days after the first specimen.
3. Heterophilic antibodies are a well-recognized cause of interference in immunoassays.
These antibodies may cross-react with reagent antibodies and generate false positive signal.
4. The final diagnosis should be based on test results in conjunction with other clinical and laboratory findings.
5. Dengue immunochromatography is a screening assay. The diagnosis of acute dengue infection should be confirmed by IgM ELISA or by other tests like Real Time PCR.

Interpretation
1. This test detects the presence of Dengue NS1 antigen & IgM, IgG antibodies to dengue virus and should not be used as sole criteria for diagnosis of dengue infection.
2. In early infections and some secondary infections, detectable levels of IgM antibodies may be testing using other clinical methods is recommended. A negative result at any time does not preclude the possibility of an early infection of Dengue virus.follow up group (Dengue virus, St. Louis encephalitis, Japanese encephalitis, West Nile and yellow fever virus) is common.

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Dengue Ns1 Antigen (card Test)

Test Description	Value(s)	Unit	Reference Range
Dengue NS1 Ag Serum, card test	Serum is Non Reactive		Serum is Non Reactive

METHOD : Immunofluorescence assay

Interpretation(s)

NEGATIVE:- No detectable dengue NS1 antigen . The result does not rule out dengue infection . If this sample is negative and dengue infection is still suspected , a follow up sample should be taken and tested using serology, no later than 14 days after the initial sample is taken.

EQUIVOCAL:- Equivocal samples should be repeated . Sample that remains equivocal after repeat testing should be repeated by an alternative method or another sample should be collected

POSITIVE:- Presence of detectable dengue NS1 antigen. Dengue serology assays should be performed on follow-up samples to confirm dengue infection.

1) The clinical diagnosis must be interpreted with clinical signs and symptoms of the patient .The result from this kit are not by themselves diagnostic and should be considered in association with other clinical data and patient symptoms.

2) Screening of the general population should not be performed. The positive predictive value depends on the likelihood of the virus being present. Testing should only be performed on patients with clinical symptoms or when exposure is suspected.

3)Serology cross-reactivity across the flavivirus group is common.

4)The performance characteristics have not been established for visual result determination.

5) All sera demonstrating a positive result by EARLY ELISA test should be referred to a reference laboratory for confirmation of dengue positivity and epidemiological recording.

6) The dengue NS1 antigen is only detected in patient serum early in the course of disease, between days 1 - 9 after onset of clinical signs.

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AUSTRALIA ANTIGEN (HBsAg)

Test Description	Value(s)	Unit	Reference Range
Sample Type SERUM			
Hepatitis B Surface Antigen (HBSAg)- Rapid Screening Serum, Immunochromatography	Non Reactive		Negative

Remark
All Reactive results must be confirmed by Neutralizing confirmatory test or by HBV DNA detection assay.

NOTE
Hepatitis B surface antigen (HBsAg) is an important viral envelope protein,which appears shortly after infection and is a key serological marker for detection and diagnosis of HBV.Clearance during treatment shows recovery and development of neutralizing antibodies (anti-HBs) occurs in 90% of the patients.due to the introduction of hepatitis B vaccination programs,the serological detection of anti-HBs has become important method for monitoring of recipients upon vaccination with synthetic and natural HbsAg.

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CREATININE

Test Description	Value(s)	Unit	Reference Range
CREATININE Jaffe 's	**1.50**	mg/dl	Adult : M 0.60 - 1.40 mg/dl : F 0.60 - 1.40 mg/dl Children : 0-1 yr - 0.04 to 0.33 mg/dl : 2-5 yr- 0.04 to 0.45 mg/dl : 6-9 yr - 0.2 to 0.52 mg/dl :10-12 yr- 0.22 to 0.59mg/dl

Medical Remarks: Reprocessed and confirmed. Kindly correlate clinically.

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BLOOD UREA LEVEL

Test Description	Value(s)	Unit	Reference Range
BLOOD UREA NITROGEN (BUN)	28.62	mg/dl	0 - 45

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